



Patient Confidentiality

General Principles

Health care professionals are under a duty to maintain the confidentiality of all information that they receive in the course of providing medical treatment and care to patients. The duty protects information created, disclosed or acquired (directly or indirectly) by health care professionals in their professional capacity.

All persons, including administrative staff, who come into contact with information as part of the health care process, also have a duty to maintain the confidentiality of that information.

The general principle is that the duty of confidence prevents the disclosure of information to individuals and organisations not involved in the particular health care process. However, not all information obtained during the professional relationship is confidential. Information in the public domain is not confidential. Trivial or useless information will similarly not fall within the scope of the duty.

The duty of confidence can arise by statute, under the common law and in equity. The duty does not end when the professional relationship with the patient has ceased. Nor does it end with the death of the patient.

There are a number of exceptions to the duty of confidence where otherwise confidential information may be disclosed to third parties. These include where the person gives consent to the disclosure, the disclosure is required or permitted by law, or there is an overriding 'public interest' which justifies the disclosure.

Any decisions as to whether confidential information should be provided to any third party in the public interest should be made only by the appropriate person at the appropriate level within the Department of Health or the Health Service Provider's health service.

If any queries arise regarding the confidentiality or disclosure of information, it is recommended that advice be sought from Legal and Legislative Services or through the Health Service Provider's usual processes for seeking advice.

WA Health employees may initiate a request for legal advice by submitting the [Request for Legal Advice or Legislative Assistance form \(WORD 40KB\)](#).

Specific requests for confidential patient information

Requests under the *Freedom of Information Act 1992*

The *Freedom of Information Act 1992* (FOI Act) provides individuals with a legally enforceable right of access to their medical records held by Western Australian Government agencies unless one or more of the exemption provisions contained in the FOI Act apply. Requests for information under

the FOI Act should be referred to the appropriate Freedom of Information coordinator. A list of freedom of information contacts is available on the [HealthyWA website](#).

Requests by patients

There is no general legal obligation requiring Health Service Providers and their staff to comply with requests by patients for a copy of their medical record or the creation of documents for non-clinical purposes. It is recommended that any such requests should be in writing with an explanation of the purpose to which the report is to be used. Legal advice should be sought from Legal and Legislative Services or through the Health Service Provider's usual processes to seek advice if there are any queries about the request.

Requests by the patient's relatives and other individuals

It should not be presumed that a patient's medical condition and details can be disclosed to or discussed with the patient's relatives. The principle of autonomy means the patient has the general right to decide who does or does not receive information about his/her treatment.

Confidential patient information should not be given to the patient's relatives without the consent (express or implied) of a competent patient or unless there is other legal basis such as a guardianship order.

Requests by solicitors

There is no general legal obligation requiring health care professionals and other individuals to comply with requests by solicitors for the creation of documents for non-clinical purposes, for example a medical report for use in legal proceedings. Confidential patient information should not be released to solicitors, whether acting on the patient's behalf or not, without a legal basis to do so or the patient's express written consent authorising the disclosure.

If a request is made by a solicitor, it is recommended that any such requests be made in writing. The request should state on whose behalf the solicitor is instructed (the person may not be the patient's solicitor), detail the request being made and explain the reason for the request.

Where there is a possibility a medical report or other document has been requested for a potential or actual claim against the health service, it is recommended that Department of Health employees seek legal advice from Legal and Legislative Services or from the Insurance Commission of WA lawyers dealing with the claim. Health Services Provider employees should seek legal advice in accordance with your internal policies.

Subpoenas and summonses

Information can be found at on the [Subpoenas and Summonses \(PDF 265KB\)](#) fact sheet and the [Appearing as a Witness in Court \(PDF 107KB\)](#) fact sheet.

Requests by health care professionals for therapeutic purposes

Health care professionals and other individuals within a health care facility who have a legitimate therapeutic interest in the care of the patient may generally have access to the information they need to know in order to provide appropriate care and treatment. Consent to the sharing of information in these circumstances will generally be implied. If there is any doubt that the information requested is for therapeutic use, seek legal advice from Legal and Legislative Services or through the Health Service Provider's usual processes to seek advice.

Requests by or on behalf of Coroners

Information can be found on the [Clinical Excellence Division Patient Safety Surveillance Unit](#) intranet page and in the [Clinical Excellence Division Review of Death Policy MP 0098/18](#).

Requests by the police

In Western Australia, there is no general legal obligation on health care professionals and other individuals to report the commission or suspected commission of an offence to the police. However, some statutes permit or require the reporting of certain offences for example under the *Children and Community Services Act 2004* a person acting in good faith may provide information to the CEO or another officer of the Department of Communities about the wellbeing of the child under section 129(1)(a) and may provide information in response to a request from the CEO of the Department of Communities under section 23(3) of that Act. Further, under the *Firearms Act 1973* where a medical practitioner or registered nurse believes a firearm or ammunition was involved in an injury for which medical assistance is sought they may report this to the Commissioner of Police under section 23B(1)(b) of that Act.

There is also no general legal obligation on health care professionals and other individuals to assist the police with their enquiries by answering questions, providing witness statements or preparing medical reports or other documentation not already in existence.

However, where assistance is given to the police, the individual providing the assistance must not give false or misleading information or take any active steps to prevent or obstruct the discovery or investigation of an offence by the police.

The police do have the power to compel the production of pre-existing documentation by means of a valid search warrant or similar court order. Consequently, the circumstances in which confidential patient information may be disclosed to the police without a breach of confidence arising include:

- Where the police produce a valid search warrant or similar court document authorising the seizure or disclosure of specified documents already in existence.
- Where the police provide the written consent of the patient (or other person with legal authority to consent on the patient's behalf such as the parents or other legal guardian of a patient who is a minor) and information is disclosed in accordance with the consent given.
- Where disclosure is made pursuant to a statutory provision.

Disclosure of confidential patient information in response to a summons to produce documents or to give evidence must be made to the court and not to the police.

Legal advice should be sought if you are unclear whether the information should be disclosed.

More information is available on the [Subpoenas and Summonses \(PDF 265KB\)](#) fact sheet.

Requests by and disclosures to the Australian Health Practitioner Regulation Agency (AHPRA)

The *Health Practitioner Regulation National Law (WA) Act 2010* (National Law) requires that certain notifiable conduct be reported to AHPRA by health practitioners and their employers. See [Notifiable and Reportable Conduct Policy MP 0125/19](#) and supporting documents for information on notifications.

A national board established under the National Law, for example the Medical Board of Australia, may appoint investigators. Investigators may give written notice requiring a person to provide specified information or attend a specified place and produce documents. If a request is received, a copy of the investigator's identity card should be obtained, and a written request stating the information sought should be provided. More information about investigations is available on the [AHPRA website](#).

Requests by researchers

Queries regarding research agreements should be directed to the appropriate officer. See the [Research Governance policy MP 0162/21](#) for further information.

Requests by the Department of Communities Child Protection and Family Support

See the [Guidelines for Protecting Children 2020](#) for further information.

Requests about patients who are minors

See the [Guidelines for Protecting Children 2020](#) for further information.

Requests by the media

See the [Use of Official Information, Digital Services \(Social Media\) and Public Comment Policy MP 0128/20](#) for further information.

Requests about deceased patients

In the case of a deceased adult patient, the deceased's personal representative such as an executor under a Will or an Administrator is the relevant person to give consent to the release of the deceased's confidential information.

However, there is a requirement to follow the third-party consultation process under the *Freedom of Information Act 1992* (FOI Act) where access to a deceased patient's confidential medical records are sought pursuant to that legislation. In those circumstances, the 'closest relative' must be consulted. The executor or administrator does not have rights to access records under the FOI Act.

Privacy Act 1988 (Commonwealth)

Application of Privacy Act to state government agencies

The *Privacy Act 1988* and *Privacy Amendment (Private Sector) Act 2000* require personal information (which includes patient information) held by Commonwealth government bodies and private sector organisations to be kept private. Private sector organisations include private hospitals, private health centres and other privately administered hospitals and health services.

The foundation of the legislation is a set of Australian Privacy Principles that set out the standards that need to be met to make the collection and use of personal information lawful.

The above legislation does not apply to State government bodies including Western Australian public health services as a matter of law. However, the State Government's policy requires that information sharing should be consistent with appropriate minimum privacy standards such as the National Privacy Principles. See Public Sector Commissioner's Circular 2014-02 Policy framework and standards for information sharing between government agencies on the [Public Sector website](#).

This fact sheet is provided as a resource to employees in the Department of Health and Health Service Providers established under section 32 of the *Health Services Act 2016*. It is intended for general use and information purposes only and should not be relied upon as a substitute for legal advice.



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